

YELLOW REPORT

Given her complete metabolic response after 4 weekly doses of rituximab, it is reasonable to **stop rituximab and observe** [1] in an 82-year-old with SMZL, as maintenance rituximab prolongs progression-free survival but has not shown an overall survival benefit and increases infection risk [2] [3]. Observation is appropriate because she has no high-risk features, and retreatment at relapse is effective [1]. If she values time-to-next-treatment over infection risk, a limited, time-defined maintenance course (e.g. rituximab 375 mg/m² IV every 2–3 months for up to 2 years) can be considered, balancing PFS benefit against cumulative B-cell depletion and infections, with a clear plan to stop at any grade ≥ 2 infusion reaction or $\geq$ grade 2 infectious complication; however, outside clinical trials, any extension beyond 2 years should generally be avoided given lack of demonstrated added benefit and incremental immunosuppression [1] [3].

Evidence supporting rituximab maintenance in SMZL

- **PFS benefit:** Retrospective data show maintenance rituximab prolongs PFS in SMZL (7-year PFS 75% with maintenance vs 39% without) [1].
- **No OS benefit:** No overall survival advantage has been demonstrated with maintenance rituximab in SMZL [1].
- **Infection risk:** Maintenance rituximab increases infection risk, particularly in older adults [3] [4].

Evidence supporting observation after complete response

- **Guideline endorsement:** NCCN and BSH list active surveillance as an appropriate option after CR in SMZL, especially in older adults or those with comorbidities [1] [5].
- **Retreatment efficacy:** Rituximab remains effective at relapse, making observation a reasonable strategy [1].
- **Quality of life:** Observation avoids ongoing treatment burden and infection risk, which is especially relevant in older adults [3].

Patient-specific factors influencing the decision

- **Age and comorbidities:** At 82 years with asthma and spina bifida, she has higher infection risk and frailty, favoring observation [3].
- **Treatment goals:** Her priorities (PFS vs infection risk, QOL) should guide the choice [6].

- **High-risk features:** CCND3/MYC/CDKN2A mutations alone are not established predictive markers for maintenance benefit in SMZL; consider observation after CR absent other high-risk features [1].

Clinical guidelines and expert consensus

NCCN and BSH endorse **active surveillance** after CR in SMZL, especially in older adults or those with comorbidities [1] [5]. Maintenance rituximab is optional and should be individualized based on patient factors and preferences [1].

Practical considerations

If choosing observation, plan **regular follow-up** every 3–6 months for 5 years, then annually, with retreatment at relapse [1]. If choosing maintenance, use rituximab 375 mg/m² IV every 2–3 months for up to 2 years, with monitoring for infections and hypogammaglobulinemia [1]. Use a time-limited course — typically 2 years — as indefinite maintenance has not demonstrated added benefit and adds cumulative B-cell depletion; extend only on an individualized basis and generally only within clinical trials [1] [3].

Summary of recommendations

Factor	Observation after complete response	Rituximab maintenance
Age and comorbidities	Appropriate for older adults with comorbidities	Higher infection risk in older adults
Treatment goals	Prioritizes QOL and minimal treatment burden	Prioritizes prolonged PFS
High-risk features	Not required for observation	Consider if high-risk features present
Clinical guidelines	Supported by NCCN and BSH	Optional and individualized
Practical considerations	Regular follow-up and retreatment at relapse	Monitor for infections and hypogammaglobulinemia

Given her CR and age, **observation after complete response** is the most appropriate approach [1]. Maintenance rituximab may be considered only if she strongly prioritizes PFS and accepts increased infection risk [1].

Sources:

1. [NCCN guidelines® insights: B-cell lymphomas 3.2025](#). Journal of the National Comprehensive Cancer Network, 2025
2. [Rituximab as maintenance therapy for patients with follicular lymphoma](#). The Cochrane Database of Systematic Reviews, 2009
3. [Too much and not enough: revisiting maintenance rituximab in indolent lymphomas](#). Haematologica, 2022
4. [Rituximab maintenance for 2 years in patients with high tumour burden follicular lymphoma responding to rituximab plus chemotherapy \(PRIMA\): a phase 3, randomised controlled trial](#). Lancet, 2011
5. [Guideline for the diagnosis and management of marginal zone lymphomas: a British society of haematology guideline](#). British Journal of Haematology, 2024
6. [One size does not fit all: who benefits from maintenance after frontline therapy for follicular lymphoma?](#). American Society of Clinical Oncology Educational Book: American Society of Clinical Oncology. Annual Meeting, 2019